

Lichen Sclerosus

Clinical Presentation

The most common symptom of lichen sclerosus is **pruritus**, which can range from mild to severe. The affected vulvar skin is fragile, and scratching often leads to painful erosions. By the time many women seek treatment, they already exhibit late-stage signs, including significant textural changes and scarring. Scarring may result in narrowing of the introitus, leading to dyspareunia or even intolerable pain during intercourse.

Lichen sclerosus typically begins with white **papules or plaques** that commonly appear first on the anterior vulva and periclitoral area. While these lesions may occasionally be smooth and waxy—especially on moist skin—the classic presentation features **hypopigmented, well-demarcated plaques** with a **cellophane paper-like texture**. The skin is generally thin and fragile, with frequent **erosions** and **purpura**. Some patients, particularly those with severe itching, develop **thickened, hyperkeratotic** skin.

Chronic Changes

With long-standing disease, progressive **scarring** and **resorption** occur. Common findings include:

- Resorption of the **labia minora**
- Scarring and fusion of the **clitoral hood**, which may bury the clitoris
- Midline agglutination (as seen in late-stage disease)

Extragenital Involvement

Extragenital lichen sclerosus affects a minority of patients and is most often found on the **upper trunk and arms**. These lesions typically occur on keratinized skin and are usually **asymptomatic**.

Autoimmune Association

An association with circulating autoantibodies has been reported. However, the only autoimmune condition with sufficient evidence to warrant laboratory testing is **hypothyroidism**.

Differential Diagnosis

Fully developed lichen sclerosus—with its characteristic **hypopigmentation**, **shiny or crinkled texture**, and **fragile skin**—is usually easily recognized. However, early or atypical cases may be subtle or mimic other dermatoses.

Complications

- **Squamous cell carcinoma (SCC) of the vulva** is a serious complication.
- Historically, 2% to 3% of untreated patients developed SCC, particularly those with chronic **hyperkeratosis** or **erosions**.
- With effective **topical glucocorticoid therapy**, the risk of malignancy is significantly reduced.

Treatment

- **Midpotency topical glucocorticoids** may relieve symptoms but often fail to normalize skin texture or prevent scarring.
- **Ultrapotent glucocorticoids**, such as **clobetasol propionate ointment**, applied once or twice daily, typically alleviate symptoms within days.
- **Several months of treatment** (usually 3–5 months) are required for maximal clinical improvement.

Maintenance and Monitoring

- Patients should be followed **monthly** during daily treatment to:
- Monitor for **therapeutic response**

- Watch for **steroid-induced side effects** (e.g., skin atrophy)
- Once improvement is achieved, the frequency of application should be gradually reduced to a **maintenance regimen**.
- **Long-term maintenance therapy** is typically necessary to prevent relapse.